

VETERINARY SERVICES FOR HOUSEHOLD PETS

Appendix H: Supporting evidence for section 10: Choice of treatments and referrals

15 October 2025

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The Competition and Markets Authority has excluded from this published version of the final report information which the inquiry group considers should be excluded having regard to the three considerations set out in section 244 of the Enterprise Act 2002 (specified information: considerations relevant to disclosure). The omissions are indicated by [X]. Some numbers have been replaced by a range. These are shown in square brackets. Non-sensitive wording is also indicated in square brackets.

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Analysis of business practices and systems which may limit or constrain the way vets give options

- 1.1 This appendix sets out the supporting evidence for our provisional finding that business practices and systems may limit or constrain the way vets give options.
- 1.2 Like other commercial entities, veterinary businesses adopt processes that help them understand and manage their business, and to measure and guide its performance and their employees. These include using key performance indicators (**KPIs**), setting targets, issuing clinical protocols and guidance, and using IT systems such as Practice Management Systems (**PMSs**).
- 1.3 These standard business practices can help veterinary businesses run efficiently and help produce good outcomes for pet owners. However, they have the potential to put pressure on, or unduly influence, vets' decisions, including about the treatment and referral options they offer. There is some evidence that such pressures are occurring in practice in some cases, and there are no regulatory safeguards to stop it happening.
- 1.4 Some veterinary businesses (especially the larger groups, but also some independents) use KPIs and targets.¹ Evidence from submissions and internal documents shows that clinical and financial KPIs appear to be common in the industry.² Some LVGs have targets around the number or proportion of in-group referrals (compared to external referrals), or appear to guide vets to an in-group referral.³ Vets in our qualitative research reported being monitored, either on an individual level or a practice level, on the following metrics: revenue generation per consultation; diagnostic work-up rate; percentage of follow-up appointments; and turnover.⁴
- 1.5 In response to our working paper on Business models, provision of veterinary advice and consumer choice which discussed the evidence set out in the preceding paragraph:
 - CVS submitted it does not have targets around the number of in-group referrals;⁵

¹ We note that not all LVGs set targets.

² A list of the KPIs LVGs track can be found in Table 2.1 of our working paper on [Business models, provision of veterinary advice and consumer choice](#), 6 February 2025.

³ For example: [redacted] LVG response to RF11, '[redacted]'; [redacted], LVG response to RF13, '[redacted]'; [redacted] LVG response to RF13, '[redacted]'; [redacted] LVG response to RF13, '[redacted]'; [redacted] LVG response to RF13, '[redacted]'; [redacted] LVG response to RF13, '[redacted]'; [redacted] LVG response to RF13, '[redacted]'; [redacted] LVG response to RF13, '[redacted]'; [redacted] LVG response to RF13, '[redacted]'; [redacted] LVG response to RF13, '[redacted]'.

⁴ A fuller list of metrics that are monitored is discussed in the qualitative research report. Performance monitoring was reported by most of the vets working in LVGs, though to differing extents, those working at independent practices reported being monitored on performance metrics much more rarely. Qualitative research with veterinary professionals, pp 61 to 62.

⁵ [CVS response to the February 2025 working papers, 21 March 2025](#), section 4.2.1.

- VetPartners submitted that it does not track the extent of outside-group verses in-group referrals;⁶ and
- Linnaeus submitted that it does not track outbound referrals from its FOPs and does not monitor or set targets for the number or proportion of in-group referrals.⁷

1.6 In LVGs' internal documents we have seen several examples of targets that have been set around clinical KPIs. Some notable ones include:

- (a) One LVG [X] had a series of [X] projects and required the practices within its group to implement at least one of these.⁸ Metrics for these projects involved increasing the number of procedures for various diagnostics and treatments, including ultrasound, dental radiography, and ophthalmology assessments.⁹
- (b) A practice owned by another LVG [X] had KPIs for the number of diagnostic appointments, x-rays and ultrasounds, lab and initial consultations seen, with a target figure.¹⁰ The KPIs were originally introduced by the previous practice owner and are no longer being tracked by the practice.¹¹
- (c) A further LVG's [X] documents [X].¹²

1.7 The IT systems used by veterinary businesses to help run their operations usually include PMSs. These may be used, for example, to record diagnoses, keep notes, and generate prescriptions, and may be a rich source of data for monitoring KPIs. They will sometimes be structured and have system architecture designed to follow diagnostic guidance or protocols.¹³

⁶ VetPartners response to the 'Business models, provision of veterinary advice and consumer choice' working paper, 21 March 2025, paragraph 5.4.

⁷ Linnaeus response to the February 2025 working papers, 21 March 2025, paragraphs 5.29 and 5.30.

⁸ [X] LVG response to RF13, '[X]'.

⁹ The same LVG [X] produced and monitored an internal reporting measure which it identified as being associated with high quality 'work-up' (diagnosis and targeted treatment) of cases. This measure monitored the level of care given when investigating and treating clinical conditions and focused on dentals, diagnostics, laboratory, and in-patient services. The LVG [X] defines the measure as 'a fairly crude measure of how well the LVG's [X] practices are investigating and treating clinical conditions in their patients', that 'tracks what percentage of the LVG's [X] total revenue is generated from services that would be undertaken as part of the investigation and subsequent treatment of a patient's condition'. LVG [X] Internal Document, '[X]'. [X] LVG response to RF13, '[X]'. [X] LVG RF13 Response, Question 19 [X].

¹⁰ LVG [X] Internal Document, [X].

¹¹ LVG [X] response to putback, September 2025.

¹² LVG [X] Internal Document, [X].

¹³ Qualitative research with veterinary professionals, pp 38-39.

Commercially-orientated business practices and systems can support, and do not necessarily contradict, delivering good outcomes for pet owners

- 1.8 The standard business practices which veterinary businesses use can help them run efficiently and can produce good outcomes for pet owners. A concern would arise if business practices conflicted with vets' obligations to provide appropriate and timely information about treatment options.
- 1.9 Submissions from LVGs indicate that they consider that their vets can exercise clinical freedom:
- (a) In response to our working papers, [redacted] LVGs submitted that their vets have full clinical autonomy and many indicated that their vets generally consider a range of factors when providing treatment and referral options.¹⁴
 - (b) All LVGs have told us that their KPIs do not incentivise or otherwise influence the treatments their vets offer.¹⁵ Some LVGs submitted that any targets they set, or their clinical guidance and protocols, are not designed to override their vets' clinical judgements or freedom.^{16, 17}
 - (c) [redacted] LVGs have said that they did not have any policies about which referral centres vets may refer to,¹⁸ or incentives relating to this.¹⁹ One LVG's [redacted] case transfer protocol indicates that '[redacted] is encouraged where possible', but that the decision to refer is 'ultimately [...] down to the decision and discretion of the vet'.²⁰
- 1.10 Some LVGs have guidance and policies in place which encourage vets to provide pet owners with appropriate and timely information about referrals and treatment options. This includes a requirement to seek informed consent when making

¹⁴ LVG [redacted] response to the 'Business models, provision of veterinary advice and consumer choice' working paper [redacted], and LVG responses to the February 2025 working papers [redacted][redacted][redacted][redacted][redacted].

¹⁵ For example: LVG [redacted] response to RFI3, Question 31, paragraph 30, p 8, [redacted] LVG response to RFI3, Question 32, paragraph 32.2, p 9, [redacted] LVG response to RFI3, Question 32, p 13, [redacted] LVG response to RFI3, Question 32, p 3, [redacted] LVG response to RFI3, Question 32, paragraph 32.4, [redacted] LVG response to RFI3, Question 31, para 31.1, p 24.

¹⁶ [Linnaeus response to the February 2025 working papers, 21 March 2025](#), paragraph 5.24(b). [IVC response to the February 2025 working papers, March 2025](#), slide 70. [Medivet response to the February 2025 working papers, 21 March 2025](#), paragraph 3.16.

¹⁷ LVG [redacted] response to RFI2 Q32, p 9. LVG [redacted] response to RFI2 Q6, p 5. LVG [redacted] response to RFI2 Q6, p 8. Guidance documents submitted by one LVG [redacted] began with the statement 'This document is intended for guidance only'. For example, [redacted] LVG response to RFI2, [redacted].

¹⁸ LVG [redacted] response to RFI17, Question 26; LVG [redacted] response to RFI17, Question 26; LVG [redacted] response to RFI9, paragraph 16.1. We note that two LVGs [redacted] and [redacted] have referral policy documents, but these LVGs have submitted that these documents do not impact on vets' clinical freedom to select the most appropriate (internal or external) referral provider. LVG [redacted] response to RFI17, Question 26; LVG [redacted] response to RFI17, Question 26.

¹⁹ CVS Issues Statement Response, p 6. [CVS.pdf](#); [redacted] LVG response to RFI3, paragraph 32.2, IVC Issues Statement Response, paragraph 5.9. [IVC_Evidensia.pdf](#); Linnaeus Issues Statement Response, paragraph 4.9. [Linnaeus.pdf](#); [redacted] LVG response to RFI9, Question 22, p 30.

[redacted] LVG response to RFI9, paragraph 14.3.

²⁰ LVG [redacted] response to RFI17, Question 26.

referrals,²¹ or a stipulation that pet owners should always be able to consider a range of reasonable treatment options.²²

- 1.11 Financial incentives based on clinical KPIs, such as treatments sold, do not seem to be a common practice. We have seen only a very small number of examples of vets being offered such incentives, and these were of low financial value.²³
- 1.12 Few vets in our qualitative research reported that performance monitoring and financial incentives had influenced their clinical decisions.²⁴ In particular, more experienced vets and those in senior roles were more likely to report ignoring any pressures from employers; for example, due to having fewer concerns over job security.²⁵

Business practices and systems have the potential to impinge on vets' clinical freedom to offer a range of treatment and referral options

- 1.13 Despite the evidence set out above that business practices and systems can support delivering good outcomes for pet owners,²⁶ we are concerned that they have the potential to impinge on vets' clinical freedom to offer a range of treatment and referral options, and there is some (albeit limited) evidence of this effect.

Certain LVGs' acquisition strategy documents show that they considered redirecting referrals in-house

- 1.14 The acquisition strategy documents from the vertically integrated LVGs (in other words those documents that assessed the case for their purchase of FOPs and related services) show that they considered redirecting referrals in-house. They

²¹ One LVG [REDACTED] defined informed consent as requiring the provision of a range of reasonable options together with their expected financial cost, and making the client aware of any conflicts of interest (including ownership links) when referring. LVG [REDACTED] RFI11 response, Question 7, Internal Document, [REDACTED]. LVG [REDACTED] RFI17 response, Question 25, Internal Document, [REDACTED].

²² Guidance from one LVG indicates that pet owners should be informed of the fee estimates associated with these options and should have the significance and main risks explained to them before they sign a consent form. LVG [REDACTED] RFI17 response, Question 16.

²³ For example:

(a) some LVGs and independents offering small one-off payments (ie £500 or under). For example, we were told that at one LVG [REDACTED] practice everyone received a £[60-80] voucher for being the practice with the highest proportion of follow up appointments compared to others in the area. Qualitative research with veterinary professionals, p 67. (Qualitative research with veterinary professionals, pp 98-99. [REDACTED] LVG response to RFI2, Question 6, paragraph 6.3. [REDACTED] LVG response to RFI3 "[REDACTED]". [REDACTED] LVG response to RFI3 "[REDACTED]". LVG [REDACTED] Internal Document, "[REDACTED]". [REDACTED] LVG response to RFI3, "[REDACTED]"; and

(b) one LVG [REDACTED] compensates FOP clinics for any work they continue to provide when they refer cases to one of the LVG's Referrals Centres (for example building a care plan around the results of scan images) - a clinic may be able to claim up to [REDACTED]% of the fee paid by a client to the LVG depending on the originating vet's degree of involvement in the case. [REDACTED] response to RFI1, Question 42, pp 17-18.

²⁴ Qualitative research with veterinary professionals, p 71.

²⁵ Qualitative research with veterinary professionals, pp 69-70. A few junior vets reported disregarding corporate guidance with the support of senior colleagues. Qualitative research with veterinary professionals, p 70.

²⁶ Section: Commercially-orientated business practices and systems can support, and do not necessarily contradict, delivering good outcomes for pet owners.

identified increased revenues from redirecting referrals to in-group sites as a plausible outcome of the transactions.^{27, 28}

- 1.15 These documents suggest that the acquiring parties considered that it would be possible to influence vets' referral behaviour. They suggest that the parties did not expect that regulatory requirements would necessarily reduce that possibility.

Veterinary businesses measure and communicate KPIs and targets with the intention of influencing staff behaviour

- 1.16 The monitoring of KPIs and targets, and the communication of results to staff, are liable to create pressure on vets to support the business in achieving them, even in the absence of financial incentives. We consider that veterinary businesses that communicate performance against KPIs to their staff do so because this communication intended to influence staff behaviour.
- 1.17 [X] LVGs provide information on some KPIs to local practice senior leadership teams. For some LVGs [X], information on KPIs is shared directly with practice leadership in periodic reviews.^{29, 30} One LVG [X] provides information on KPIs to practices through a central online dashboard and, we understand, each practice can compare their FOP's scores against others in the region.³¹ Another LVG [X] indicated that [X].³²
- 1.18 LVG documents often show an intention to engage with vets or practices to improve performance, for example where they are not meeting targets or objectives.³³ There is academic research that suggest these soft interventions are effective in changing behaviour.³⁴

²⁷ A discussion of this evidence is in our working paper on [Business models, provision of veterinary advice and consumer choice](#), 6 February 2025, paragraphs 3.26-3.27

²⁸ [X] submitted that the potential upsides identified did not form part of the economic business case for the acquisition. [X] response to [X] working papers, [X].

²⁹ LVG [X] RFI17 response, Question 20; LVG [X] RFI17 response, Question 20; LVG [X] RFI17 response, Question 20.

³⁰ VetPartners submitted that it deliberately does not share the KPIs tracked by central management with practice staff, but business development directors have the option to share these with practice leadership teams. [VetPartners' response to the 'Business models, provision of veterinary advice and consumer choice' working paper, 21 March 2025](#), paragraph 6.11.

³¹ LVG [X] RFI17 response, Question 20.

³² LVG [X] RFI17 response, Question 20.

³³ [X] LVG response to RFI3, internal document, '[X]', March 2024, slide 5. [X] LVG response to RFI3, internal document, '[X]', February 2024, p 2. [X] LVG Internal Document, '[X]'. [X] LVG response to RFI3, internal document, '[X]'.

³⁴ Academic evidence suggests that soft interventions such as reminders, conversational accountability and social pressure are effective in bring about behavioural change. This evidence comes from various contexts such as organisational behaviour, tax compliance, public health, finance and personal goals. Thaler, R.H. and Sunstein, C.R., 2021. *Nudge: The final edition*. Penguin. Andersson, H., Engström, P., Nordblom, K. and Wanander, S., 2023. Nudges and threats: soft versus hard incentives for tax compliance. *Economic Policy*, 38(116), pp 771-819. Herath, T. and Rao, H.R., 2009. Encouraging information security behaviors in organizations: Role of penalties, pressures and perceived effectiveness. *Decision support systems*, 47(2), pp154-165.

- 1.19 Where these KPIs and targets are inter-related with clinical decisions (for example, the number of treatments sold), they have the potential to affect, or create the perception that they affect, vets' clinical decisions and their approach to engaging with customers.

We have seen some limited evidence of the pressure from KPIs, targets, guidance and IT systems affecting vets' decision-making

- 1.20 As set out above, KPIs and targets can have the effect of changing behaviour, even with only soft interventions. We have seen some evidence that this may be happening with vets. In particular:
- (a) A few vets in our qualitative research described feeling that targets might affect others' decision making.³⁵ Vets assumed that not meeting targets might compromise salary increases or promotion opportunities.³⁶
 - (b) Most of the vets working at one LVG [redacted] who participated in our qualitative research said they were questioned on targets and that management highlighted where the targets were not being met.³⁷ Similar pressure was reported by some vets at other LVGs, but by very few vets working at independent practices.³⁸ Several vets described actively seeking employment in independent practices to avoid the performance pressure they had encountered in previous roles at LVGs.³⁹
 - (c) Some individual vets have contacted us directly to say that they have felt pressure to deliver on KPIs and targets.
 - (d) After the RCVS highlighted its confidential reporting line in a press release in May 2025,⁴⁰ it received a few reports from veterinary professionals about a lack of clinical freedom. One vet reported receiving messages and being 'reprimanded' whenever they sent bloods for screening at external laboratories, rather than in-house, even though the external laboratories were cheaper for the pet owner.⁴¹ Two vets said they were required to refer within the same group.⁴² Other reports included topics such as:

³⁵ Qualitative research with veterinary professionals, pp 71-72.

³⁶ We note that many vets in the research reported that they were meeting their targets and so were not sure what would happen if they did not meet them. Qualitative research with veterinary professionals, p 71.

³⁷ Qualitative research with veterinary professionals, pp 64-65.

³⁸ Qualitative research with veterinary professionals, pp 62-63.

³⁹ Qualitative research with veterinary professionals, p 63.

⁴⁰ The RCVS website, [Statement from RCVS President on BBC Radio 4 investigation - Professionals](#). Last accessed 5 August 2025.

⁴¹ The RCVS RFI6 response, Question 7.

⁴² The RCVS RFI6 response, Question 7.

- general concerns about financial targets, which were felt to be at odds with animal welfare;
- concerns about being told which medicines to use (specifically in relation to analgesia) or restrictions on the types of anti-parasitics that they were permitted to sell; and
- being compelled to insist on vaccines being sold to clients.⁴³

- 1.21 The existence of treatment protocols, guidance or other messaging in LVGs may influence vet behaviour, including on the options provided.⁴⁴ It is therefore important that these documents are well-designed and do not conflict with vets' obligations to provide treatment options and appropriate and timely information about them. A very small number of vets have told us of situations where this type of conflict has arisen,⁴⁵ but we have been told some vets feel able to resist following protocols.⁴⁶
- 1.22 Some vets felt that the IT systems used by vet practices, such as their PMSs, can limit or constrain (either unintentionally or by design) the treatments vets offer to pet owners and for which vets charge.
- 1.23 Some participants in our qualitative research reported that the systems prompted them to comply with diagnostic guidance or protocols.⁴⁷ Most commonly, this related to how to charge for diagnostics. For example, some participants described practice protocols which required certain 'bundles' of diagnostic tests for suspected conditions, and that computer systems made it hard to charge for diagnostic tests individually, even when the vet considered that the full bundle was unnecessary in the circumstances.⁴⁸ A few vets working at LVGs reported that their practice's IT systems encouraged referrals within their own group by offering shortcuts, which made the referral process faster and easier.⁴⁹ Although shortcuts

⁴³ The RCVS RFI6 response, Question 5.

⁴⁴ One senior staff member at a veterinary school said that the practice type and their associated guidelines would shape how a vet worked and what level of treatment options were presented to consumers. Summary of roundtable with academics, paragraph 11. [Summary of roundtable discussion held with senior representatives of veterinary schools via MS Teams on 16 September 2024](#). One senior vet at a charity said that, in their experience, the large corporate groups had very rigid protocols that vets must follow. Summary of roundtable with charities, paragraph 7. [Summary of roundtable discussion held with senior veterinary professionals who currently work in the charity sector via MS Teams on 19 September 2024](#).

⁴⁵ One vet said that, while working for an LVG, there were limitations on their clinical decisions in prescribing medicines which were not on preferred product lists. Summary of Manchester roundtable, paragraph 10. [Summary of roundtable discussion held in Manchester on 28 August 2024](#). A few locum veterinary surgeons reported feeling pressure to adhere to practice guidance (such as referring internally) due to concerns that non-compliance could jeopardise their chances of securing future shifts. Qualitative research with veterinary professionals, p 70.

⁴⁶ During the roundtable with vets working in the LVGs, two attendees noted reluctance to work to protocols, and said that if a protocol directed the vet to do something contrary to the best interest of the pet or its owner or professional obligations, they would not follow it. [Summary of vets who work at large corporate groups roundtable discussions](#), paragraph 13.

⁴⁷ Qualitative research with veterinary professionals, pp 38-39.

⁴⁸ Qualitative research with veterinary professionals, p 6.

⁴⁹ Qualitative research with veterinary professionals, p 52.

can save time for vets, the presence of a default option could subtly influence vets to refer within the same group.⁵⁰

- 1.24 In addition to evidence of the pressure from KPIs, targets, guidance and IT systems affecting vets' decision-making, there is evidence that vets and non-vets may be under pressure to recommend pet owners attend OOH consultations rather than giving advice over the phone. In response to the RCVS press release referred to above, one vet told the RCVS that they were told that phone calls between 1am and 6am were actively monitored and that they were not supposed to give free advice, for example if the pet owner was not sure whether to bring in their pet. They said they were 'threatened with disciplinary action' if they did not 'actively' encourage clients to come to the clinic.⁵¹

⁵⁰ Johnson, E.J. and Goldstein, D., 2003. Do defaults save lives?. *Science*, 302(5649), pp 1338-1339.
Jachimowicz, Johnson, E.J., Bellman, S. and Lohse, G.L., 2002. Defaults, framing and privacy: Why opting in-opting out. *Marketing letters*, 13, pp 5-15.
J.M., Duncan, S., Weber, E.U. and Johnson, E.J., 2019. When and why defaults influence decisions: A meta-analysis of default effects. *Behavioural Public Policy*, 3(2), pp 159-186.
Loewenstein, G., Bryce, C., Hagmann, D. and Rajpal, S., 2015. Warning: You are about to be nudged. *Behavioral Science & Policy*, 1(1), pp 35-42.

⁵¹ The RCVS RFI6 response, Question 7.